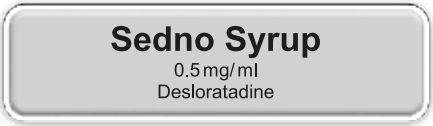


Size: L 210 x W 160 mm (Both side print)
After folding : 160 x 36 mm



NAME OF PRODUCT
Sedno Syrup 0.5 mg/ ml

NAME AND STRENGTH OF ACTIVE INGREDIENT(S)
Each 5 ml contains Desloratadine 2.5 mg.

PRODUCT DESCRIPTION
A pink color clear liquid having blackcurrant flavor and sweet taste; free from any visible foreign particles.

PHARMACODYNAMICS
Mechanism of action
Desloratadine is a non-sedating, long-acting histamine antagonist with selective peripheral H1-receptor antagonist activity. After oral administration, desloratadine selectively blocks peripheral histamine H1-receptors because the substance is excluded from entry to the central nervous system. Desloratadine has demonstrated antiallergic properties from in vitro studies. These include inhibiting the release of proinflammatory cytokines such as IL-4, IL-6, IL-8, and IL-13 from human mast cells/ basophils, as well as inhibition of the expression of the adhesion molecule P-selectin on endothelial cells. The clinical relevance of these observations remains to be confirmed.

Clinical efficacy and safety
Paediatric population
Efficacy of desloratadine oral solution has not been investigated in separate paediatric trials. However, the safety of desloratadine syrup, which contains the same concentration of desloratadine as desloratadine oral solution, was demonstrated in paediatric trials. Children, 1-11 years of age, who were candidates for antihistamine therapy received a daily desloratadine dose of 1.25 mg (1 through 5 years of age) or 2.5 mg (6 through 11 years of age). Treatment was well tolerated as documented by clinical laboratory tests, vital signs, and ECG interval data, including QTc. When given at the recommended doses, the plasma concentrations of desloratadine were comparable in the paediatric and adult populations. Thus, since the course of allergic rhinitis/chronic idiopathic urticaria and the profile of desloratadine are similar in adults and paediatric patients, desloratadine efficacy data in adults can be extrapolated to the paediatric population. Efficacy of desloratadine syrup has not been investigated in paediatric trials in children less than 12 years of age.

Adults and adolescents
Desloratadine does not readily penetrate the central nervous system. In adult and adolescent patients with allergic rhinitis, desloratadine tablets were effective in relieving symptoms such as sneezing, nasal discharge and itching, as well as ocular itching, tearing and redness, and itching of palate. Desloratadine effectively controlled symptoms for 24 hours. The efficacy of desloratadine tablets has not been clearly demonstrated in trials with adolescent patients 12 through 17 years of age. In addition to the established classifications of seasonal and perennial, allergic rhinitis can alternatively be classified as intermittent allergic rhinitis and persistent allergic rhinitis according to the duration of symptoms. Intermittent allergic rhinitis is defined as the presence of symptoms for less than 4 days per week or for less than 4 weeks. Persistent allergic rhinitis is defined as the presence of symptoms for 4 days or more per week and for more than 4 weeks. Desloratadine tablets were effective in alleviating the burden of seasonal allergic rhinitis as shown by the total score of the rhino- conjunctivitis quality of life questionnaire. The greatest amelioration was seen in the domains of practical problems and daily activities limited by symptoms. Chronic idiopathic urticaria was studied as a clinical model for urticarial conditions, since the underlying pathophysiology is similar, regardless of etiology, and because chronic patients can be more easily recruited prospectively. Since histamine release is a causal factor in all urticarial diseases, desloratadine is expected to be effective in providing symptomatic relief for other urticarial conditions, in addition to chronic idiopathic urticaria, as advised in clinical guidelines.

PHARMACOKINETICS
Absorption

Desloratadine plasma concentrations can be detected within 30 minutes of desloratadine administration in adults and adolescents. Desloratadine is well absorbed with maximum concentration achieved after approximately 3 hours; the terminal phase half-life is approximately 27 hours. The degree of accumulation of desloratadine was consistent with its half-life (approximately 27 hours) and a once daily dosing frequency. The bioavailability of desloratadine was dose proportional over the range of 5 mg to 20 mg.

Distribution
Desloratadine is moderately bound (83 % - 87 %) to plasma proteins. There is no evidence of clinically relevant active substance accumulation following once daily adult and adolescent dosing of desloratadine (5 mg to 20 mg) for 14 days.

Biotransformation
The enzyme responsible for the metabolism of desloratadine has not been identified yet, and therefore, some interactions with other medicinal products cannot be fully excluded. Desloratadine does not inhibit CYP3A4 in vivo, and in vitro studies have shown that the medicinal product does not inhibit CYP2D6 and is neither a substrate nor an inhibitor of P-glycoprotein.

Elimination
In a single dose trial using a 7.5 mg dose of desloratadine, there was no effect of food (high-fat, high caloric breakfast) on the disposition of desloratadine. In another study, grapefruit juice had no effect on the disposition of desloratadine.

Renally impaired patients
The pharmacokinetics of desloratadine in patients with chronic renal insufficiency (CRI) was compared with that of healthy subjects. The exposure to desloratadine was approximately 2 and 2.5-fold greater in subjects with mild to moderate and severe CRI, respectively, than in healthy subjects after single dose. In multiple-dose, steady state was reached after Day 11, and compared to healthy subjects the exposure to desloratadine was ~1.5-fold greater in subjects with mild to moderate CRI and ~2.5-fold greater in subjects with severe CRI.

INDICATION
Sedno Syrup is indicated for the rapid relief of symptoms associated with allergic rhinitis, such as sneezing, nasal discharge and itching, congestion/ stuffiness, as well as ocular itching, tearing and redness.

Sedno Syrup is also indicated for the relief of symptoms associated with urticaria such as the relief of itching and the size and number of hives.

RECOMMENDED DOSAGE
Adults and children ≥12 years 10 mL (5 mg) once a day. Children 6-11 years: 5 mL (2.5 mg) once a day; 1-5 years: 2.5 mL (1.25 mg) once a day. All doses can be taken with or without a meal. The prescriber should be aware that most cases of rhinitis below 2 years of age are of infectious origin and there are no data supporting the treatment of infectious rhinitis with Sedno Syrup.

ROUTE OF ADMINISTRATION
Oral

CONTRAINDICATION
Hypersensitivity to Desloratadine or to any of the excipients of Sedno Syrup.

WARNING AND PRECAUTION
Paediatric population
In children below 2 years of age, the diagnosis of allergic rhinitis is particularly difficult to distinguish from other forms of rhinitis. The absence of upper respiratory tract infection or structural abnormalities, as well as patient history, physical examinations, and appropriate laboratory and skin tests should be considered. Approximately 6 % of adults and children 2- to 11-year old are phenotypic poor metabolisers of desloratadine and exhibit a higher exposure. The safety of desloratadine in children 2- to 11-years of age who are poor metabolisers is the same as in children who are normal metabolisers. The effects of desloratadine in poor metabolisers < 2 years of age have not been studied. In the case of severe renal insufficiency, desloratadine oral solution should be used with caution.

This medicinal product contains sorbitol. Patients with rare hereditary problems of fructose intolerance should not take this medicine.
Effects on ability to drive and use machines
Desloratadine oral solution has no or negligible influence on the ability to drive and use machine. Patients should be informed that most people do not experience drowsiness. Nevertheless, as there is individual variation in response to all medicinal products, it is recommended that patients are advised not to engage in activities requiring mental alertness, such as driving a car or using machines, until they have established their own response to the medicinal product.

INTERACTIONS WITH OTHER MEDICAMENTS
No clinically relevant interactions were observed with desloratadine tablets in which erythromycin or ketoconazole were co-administered.

Paediatric population
Interaction studies have only been performed in adults. Desloratadine tablets taken concomitantly with alcohol did not potentiate the performance impairing effects of alcohol. However, cases of alcohol intolerance and intoxication have been reported. Therefore, caution is recommended if alcohol is taken concomitantly.

PREGNANCY AND LACTATION
Pregnancy
A large amount of data on pregnant women indicate no malformative nor foeto/neonatal toxicity of desloratadine. Animal studies do not indicate direct or indirect harmful effects with respect to reproductive toxicity. As a precautionary measure, it is preferable to avoid the use of desloratadine oral solution during pregnancy.

Breast-feeding
Desloratadine has been identified in breastfed newborns/infants of treated women. The effect of desloratadine on newborns/infants is unknown. A decision must be made whether to discontinue breast-feeding or to discontinue/abstain from desloratadine therapy taking into account the benefit of breast feeding for the child and the benefit of therapy for the woman.

SIDE EFFECTS
Summary of the safety profile
Paediatric population
In paediatric population, the desloratadine syrup formulation was administered to children aged 6 months through 11 years. The overall incidence of adverse events in children 2 through 11 years of age was similar. In infants and toddlers aged 6 to 23 months, the most frequent adverse events reported in were diarrhoea, fever and insomnia. No adverse events were seen in children between 6 and 11 years of age following a single 2.5 mg dose of desloratadine oral solution.

Adults and adolescents
At the recommended dose, in adults and adolescents in a range of indications including allergic rhinitis and chronic idiopathic urticaria, undesirable effects with desloratadine were reported. The most frequent of adverse events reported were fatigue, dry mouth and headache.

Tabulated list of adverse reactions
Frequencies are defined as very common, common, uncommon, rare, very rare and not known.

System Organ Class	Frequency	Adverse reactions seen with desloratadine oral solution
Psychiatric disorders	Very rare	Hallucinations
Nervous system disorders	Common Common (children less than 2 years) Very rare	Headache Insomnia Dizziness, somnolence, insomnia, psychomotor hyperactivity, seizures
Cardiac disorders	Very rare Not known	Tachycardia, palpitations QT prolongation

Gastrointestinal disorders	Common Common (children less than 2 years) Very rare	Dry mouth Diarrhoea Abdominal pain, nausea, vomiting, dyspepsia, diarrhoea
Hepatobiliary disorders	Very rare Not known	Elevations of liver enzymes, increased bilirubin, hepatitis Jaundice
Skin and subcutaneous skin disorders	Not known	Photosensitivity
Musculoskeletal and connective tissue disorders	Very rare	Myalgia
General disorders and administration site conditions	Common Common (children less than 2 years) Very rare Not known	Fatigue Fever Hypersensitivity reactions (such as anaphylaxis, angioedema, dyspnoea, pruritus, rash, and urticaria) Asthenia

Paediatric population
Other undesirable effects reported in paediatric patients with an unknown frequency included QT prolongation, arrhythmia and bradycardia.

SYMPTOMS AND TREATMENT OF OVERDOSE
The adverse event profile associated with overdosage is similar to that seen with therapeutic doses, but the magnitude of the effects can be higher.
Treatment
In the event of overdose, consider standard measures to remove unabsorbed active substance. Symptomatic and supportive treatment is recommended. Desloratadine is not eliminated by haemodialysis; it is not known if it is eliminated by peritoneal dialysis.
Symptoms
Based on a multiple dose in adults and adolescents, in which up to 45 mg of desloratadine was administered (nine times the clinical dose), no clinically relevant effects were observed.

Paediatric population
The adverse event profile associated with overdosage is similar to that seen with therapeutic doses, but the magnitude of the effects can be higher.

STORAGE CONDITION
Store below 30°C. Protect from light. Keep out of the reach of children.

DOSAGE FORM & PACKAGING AVAILABLE
Syrup. 60ml per bottle

THERAPEUTIC CODE
R06AX27 - Desloratadine; Belongs to the class of other antihistamines for systemic use.

MANUFACTURED BY
SQUARE PHARMACEUTICALS PLC.
Square Road, Salgaria, Pabna 6600, Bangladesh

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